Introduction to medical ethics

Morality, law and deontology · ethical theories · principles · codes · the Bulgarian framework · case analysis

Practical Class 1 · Social Medicine and Medical Ethics I · 2026/2027

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Registration



The course and how to work in it

Format

- 7 practical classes in medical ethics, 90 minutes each, plus lectures
- two progress tests with bonus points; examination in January and a resit
- final state examination in Social Medicine and Medical Ethics, Year 6

Resources

- examination syllabus (English)
- compendium for the state examination
- lecturer's website: slides, 15-minute readings, learning tasks with model answers
- textbook of medical ethics (University bookshop)

Every class follows the same logic: the ethical problem → the arguments → the Bulgarian legal rule → a case you decide and justify.

Learning outcomes for today

By the end of this class you should be able to:

1. distinguish **morality, ethics, law and deontology** and explain why a lawful act may still be unethical;
2. apply **four ethical theories** — consequentialism, deontology, virtue ethics, care ethics — to the same clinical problem;
3. define the **four principles** of biomedical ethics, explain **specification and balancing**, and state the main criticisms of principlism;
4. trace the line **Hippocratic Oath → Percival → Nuremberg → Geneva → Helsinki → Oviedo** and say what each document added;
5. name the **Bulgarian sources** that bind a physician — Constitution, ratified treaties, Health Act, the Act on the professional organisations, the Code of Professional Ethics — and who enforces them;
6. analyse a short case with a **structured method** and defend a decision.

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2. Ethical theories
3. Principlism and its critics
4. From Hippocrates to Oviedo
5. The Bulgarian regulatory framework
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Morality and ethics are not the same thing

Morality — the beliefs, rules and values by which a group judges conduct as right or wrong. It is learned through family, culture and religion and has **value-orienting, regulatory and formative** functions.

Ethics — the **systematic, critical reflection** on morality: it asks which norms are justified and why.

Level	**Question**
Descriptive ethics	What do physicians and patients actually believe and do? (empirical)
Normative ethics	What **ought** to be done, and on what grounds?
Applied ethics	What follows for a concrete field — medicine, research, public health?
Metaethics	What do "good", "right", "duty" mean; can moral claims be true?

Medical ethics is normative and applied: it must end in a justified decision, not only in a description of opinions.

Ethics and law: overlapping, not identical

	Law	Ethics
Source	state authority: parliament, government, courts	reasoned argument, conscience, professional community
Form	written, published, hierarchical	written (codes) and unwritten
Sanction	state coercion: fines, imprisonment, damages, loss of licence	moral criticism; professional sanction when codified
Scope	one jurisdiction, at a given time	claims general validity
Standard	enforceable requirements	can support, exceed or criticise the law

Lawful but ethically questionable

A physician accepts a generously paid place on a pharmaceutical company's "advisory board" involving almost no real work, declares it as the formal rules require — and then prescribes the company's product.

Law that was ethically wrong

Eugenic sterilisation laws of the 20th century (e.g. Buck v. Bell, USA, 1927) show that legality does not guarantee moral acceptability.

Deontology: two meanings of one word

Medical deontology — professional duties

From Greek **deon** (duty) and **logos**. The term was used by J. Bentham (**Deontology, or the Science of Morality**, 1834). In the Central and Eastern European tradition **medical deontology** is the doctrine of the physician's professional duties: conduct towards patients, colleagues and society, confidentiality, collegiality, competence.

Deontological ethics — a moral theory

A family of theories (Kant, W. D. Ross) holding that the rightness of an act depends on **duties and rights**, not only on consequences. Some acts — deceiving, using a person merely as a means — are wrong even when they would produce good results.

Medical ethics	Bioethics	Medical deontology	Medical law
moral problems of clinical practice, centred on the physician–patient relationship	ethics of medicine **and** life sciences, research, technology, environment, policy	doctrine of the physician's professional duties — partly codified	binding legal rules on health care, rights and liability

Bioethics: a young name for old questions

- **1927** — Fritz Jahr uses **Bio-Ethik** for the moral attitude towards humans, animals and plants.
- **1970–1971** — Van Rensselaer Potter independently coins **bioethics**; **Bioethics: Bridge to the Future** (1971) calls for a bridge between biological knowledge and human values.
- **1970s** — research scandals, dialysis allocation, transplantation and intensive care give bioethics its biomedical focus; the field becomes interdisciplinary: medicine, philosophy, law, theology, social sciences.

Today's agenda

genetics and genome editing · assisted reproduction · transplantation · end of life · research with humans · artificial intelligence and data · pandemics and public health · climate and health

Medical ethics asks "what should this doctor do for this patient?"; bioethics also asks "what should society allow medicine and science to do?"

Five kinds of "ought" in a clinical decision

Kind of norm	Example in Bulgaria	Binding force
Ethical argument	"Deceiving the patient violates respect for persons"	force of reasons
Legal rule	Health Act, Art. 87(1): medical activities are carried out after informed consent	state sanction
Professional rule	Code of Professional Ethics, Art. 26; rules of good medical practice	sanction by the Bulgarian Medical Association under statute
Institutional policy	hospital rules on visitors, documentation, consent forms	employment discipline; must comply with law
Personal conviction	religious or moral views of the individual physician	a personal moral reason; does not override professional and legal duties and does not bind the patient

Good reasoning names which kind of "ought" it uses and checks that a lower level does not contradict a higher one. An **ethical problem** asks what is right; a **dilemma** exists when two moral requirements cannot both be fulfilled — distinguish both from factual uncertainty.

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Consequentialism: judge acts by their outcomes

Utilitarianism (J. Bentham, 1789; J. S. Mill, 1861): the right act is the one that produces the **greatest overall well-being**, counting everyone's interests equally.

- **Act** utilitarianism — evaluate each individual act.
- **Rule** utilitarianism — follow the rules whose general observance gives the best results (e.g. confidentiality kept even when one breach looks useful).

In medicine: priority setting, cost-effectiveness analysis, vaccination policy, triage.

Strengths

impartial; attentive to real effects; influential in public health and the allocation of scarce resources

Weaknesses

may justify sacrificing an individual for the many; consequences are uncertain; the **sum** of benefit says nothing about **who** bears the burden

Deontology: duties, rights and dignity

I. Kant (1785) — the categorical imperative

- **Universal law:** act only on a maxim you could will to become a universal law.
- **Humanity:** treat every person always as an end and **never merely as a means**.

Consequence for medicine: no deception "for the patient's good"; no use of a patient for teaching or research without consent.

W. D. Ross (1930) — prima facie duties

Several independent duties — fidelity, reparation, gratitude, justice, beneficence, self-improvement, non-maleficence. Each is **prima facie** — binding but not absolute ("at first sight"): it holds **unless outweighed** by a stronger duty in the concrete situation.

Strength

protects the individual against being sacrificed for aggregate benefit; grounds rights and informed consent

Weakness

duties conflict (truthfulness vs. preventing harm) and strict rules can ignore context

Virtue ethics and the ethics of care

Virtue ethics (Aristotle)

Asks **"what kind of physician should I be?"** The right act is what a person of good character with **practical wisdom** (*phronesis*) would do. Medical virtues: trustworthiness, compassion, honesty, courage, justice, prudence, integrity.

Limit: gives little concrete guidance in a new dilemma; "a good doctor would know" is not an argument.

Ethics of care (C. Gilligan, 1982; N. Noddings)

Morality grows from **relationships of dependence** — attentiveness, responsiveness, responsibility for the vulnerable. Persons are seen in their families and communities (**relational autonomy**).

Limit: risk of partiality towards those close to us and of excusing paternalism inside the family; fair distribution of care needs a separate justification.

Other approaches you will meet: **casuistry** — reasoning from paradigm cases (Jonsen & Toulmin, 1988); **narrative ethics** — the patient's life story gives meaning to the choice; **communitarianism** — shared goods and solidarity.

One case through four lenses

Situation

A 34-year-old man has a newly confirmed HIV infection; he has not started antiretroviral therapy and his viral load is not suppressed. He asks his physician not to tell his wife, who is also the physician's patient. He refuses to use condoms "for now".

Lens	The question it makes us ask
Consequentialism	What are the likely harms to the wife and to the patient's future engagement with care? What happens to public trust if doctors disclose easily — or never?
Deontology	What duty of confidentiality was promised? Does the wife have a right not to be exposed to serious harm? Is anyone used merely as a means?
Virtue ethics	What would an honest, courageous and compassionate physician do — and how would they try to persuade the patient first?
Care ethics	Which relationships are at stake and how can they be protected — counselling, time, support for disclosure by the patient himself?

The Bulgarian rules on medical confidentiality and its limits are the topic of Class 3.

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Principlism: four mid-level principles

Respect for autonomy

respect the informed, voluntary decisions of persons capable of making them; support those whose capacity is limited

Beneficence

act for the benefit of the patient; weigh benefits against risks and costs in the light of the patient's own goals

Non-maleficence

do not inflict unnecessary or disproportionate harm (*primum non nocere*); harm must be justified by expected benefit

Justice

fair distribution of benefits, risks and costs; like cases treated alike; no discrimination

T. L. Beauchamp and J. F. Childress, **Principles of Biomedical Ethics** (1st ed. 1979; 8th ed. 2019). The principles are **prima facie** — binding but not absolute: each binds unless it conflicts with an equal or stronger obligation in the concrete case. They are not ranked in advance.

Specification — narrowing a principle for a context: "respect for autonomy includes the right of an adult with capacity to refuse life-sustaining treatment".

Balancing — deciding which obligation weighs more here; the infringement must be necessary, minimal, and openly justified.

Inside the principles: what they demand and what they do not

Principle	Demands	Does not mean
Autonomy	information, understanding, voluntariness, capacity **for this decision**; respect for refusal	that the patient can demand any treatment; that a diagnosis alone removes capacity
Beneficence	positive action: treat, prevent, rescue, relieve; balance benefit and burden	deciding **for** a capable patient what is good for him (hard paternalism)
Non-maleficence	due care; avoid unnecessary risk; proportionality of risk to benefit	avoiding every risk — no effective treatment is risk-free
Justice	non-discrimination; fair procedures for scarce resources; attention to the worst off	identical treatment for everyone regardless of need

Soft paternalism protects a person whose particular choice is shown, on assessment, not to be autonomous (e.g. in delirium); **hard** paternalism overrides an autonomous choice and needs exceptional justification. Capacity assessment: Class 2.

Justice: how should health care be distributed?

Theory	Criterion of a fair distribution
Egalitarian (J. Rawls)	fair equality of opportunity; inequalities only if they benefit the worst off
Utilitarian	maximise total health gained per resource (e.g. quality-adjusted life-years, QALYs)
Libertarian (R. Nozick)	entitlement through just acquisition and voluntary transfer; no general redistributive right to health care
Communitarian	shared values of the community define what is owed

Course classification (textbook of medical ethics)

1. status-based proportionality (historical)
2. social justice — basic goods for all
3. utilitarian maximisation
4. a guaranteed minimum, then allocation by contribution
5. perfect justice as an ideal we approach

> *Citizens shall have the right to health insurance guaranteeing them affordable medical care, and to free use of medical services under conditions and procedures determined by law.*
>
> — Constitution of the Republic of Bulgaria, Art. 52(1)

Critiques of principlism

No method for conflicts

Clouser & Gert (1990): the principles are "chapter headings" that remind us of themes but do not tell us what to do when they collide; balancing can hide intuition.

A European alternative

Rendtorff (2002), Barcelona Declaration (1998): **autonomy, dignity, integrity, vulnerability** — more relational and closer to European law (Oviedo, Art. 1: dignity and identity of the human being).

Defence and refinement

Gillon (1994; 2003): four principles **plus attention to scope** — to whom do we owe these duties? — with respect for autonomy as "first among equals".

Relational and cultural critique

Care and virtue ethics: principles neglect relationships and character. In family-centred cultures decisions are often **shared** — but sharing a decision is not the same as taking it away from the patient.

Clouser & Gert, J Med Philos 1990;15:219–236 · Gillon, BMJ 1994;309:184 · Gillon, J Med Ethics 2003;29:307–312 · Rendtorff, Med Health Care Philos 2002;5:235–244

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The Hippocratic tradition: lasting core, clear limits

The Hippocratic Oath (c. 4th century BCE) obliges the physician to:

- honour the teacher and pass on the art;
- use treatment to help the sick "according to my ability and judgment", never to injure;
- give no deadly drug and no abortive pessary;
- abstain from injustice and sexual relations with patients and their households;
- keep silent about what is seen and heard in treatment.

Primum non nocere is a later Latin formula, not a line of the Oath.

What survives

beneficence, non-maleficence, confidentiality, professional integrity, duties beyond self-interest

What is missing

the **patient's consent** — the physician decides "by my judgment"; no concept of justice in access; a guild oath sworn to gods, not a public, accountable standard

Before 1945: from virtue of the healer to codes of the profession

Period	Key development
c. 1750 BCE	Code of Hammurabi: fees and penalties of the healer depend on the outcome and the patient's social status
4th c. BCE	Hippocratic Oath and Corpus: benefit, no harm, confidentiality
4th–10th c.	Christian hospitals (Basil the Great, Caesarea); Byzantine and Islamic hospitals; al-Ruhawi, *Ethics of the Physician*
1803	Thomas Percival, *Medical Ethics* — duties in the hospital, to colleagues and to society; basis of the first AMA Code (1847)
1846–1864	the public demonstration of ether anaesthesia (1846) speeds its spread and makes relief of suffering an achievable duty; Red Cross (1863) and First Geneva Convention (1864): care for the wounded of any side
1901	first congress of Bulgarian physicians — foundation of the Bulgarian Medical Association

Nuremberg: the turning point

The Doctors' Trial (Nuremberg, Dec 1946 – Aug 1947): 23 defendants, 20 of them physicians; 16 convicted, 7 sentenced to death. Crimes: lethal experiments on prisoners (hypothermia, high altitude, infections, sterilisation) and the killing of disabled people.

The judgment formulated **ten principles** — the **Nuremberg Code** (1947).

> *The voluntary consent of the human subject is absolutely essential.*
>
> — Nuremberg Code, principle 1

Other principles: scientific necessity, prior animal studies, avoidance of unnecessary suffering, risk proportionate to benefit, qualified investigators, **freedom to withdraw**.

Abuse did not end in 1945

Tuskegee study, USA (1932–1972): treatment for syphilis was withheld from Black men to observe the disease. The scandal led to the **Belmont Report** (1979): respect for persons, beneficence, justice.

Shuster E. Fifty years later: the significance of the Nuremberg Code. N Engl J Med 1997;337:1436–1440

International standards after 1945

Document	Year	What it adds
WMA Declaration of Geneva	1948 / 2017	modern physician's pledge; the 2017 revision adds **respect for autonomy and dignity**, sharing knowledge and care for the physician's own health
WMA International Code of Medical Ethics	1949 / 2022	general duties to patients, colleagues and society
WMA Declaration of Helsinki	1964 / 2024	research with humans: ethics committee review, informed consent, vulnerable groups, placebo, post-trial access
Belmont Report (USA)	1979	respect for persons, beneficence, justice in research
WMA Declaration of Lisbon	1981 / 1995	rights of the patient — expressly referred to in the Bulgarian Code, Art. 10
Oviedo Convention (Council of Europe)	1997	first **binding** international treaty on biomedicine

Parsa-Parsi RW. JAMA 2017;318:1971–1972 · WMA Declaration of Helsinki, JAMA 2025;333:71–74

The Oviedo Convention and its place in Bulgarian law

Convention on Human Rights and Biomedicine (Council of Europe, CETS No. 164): opened for signature 04.04.1997, in force 01.12.1999. **Bulgaria:** signed 31.05.2001; ratification act of 26.02.2003 (State Gazette 21/2003); instrument deposited 23.04.2003; in force 01.08.2003.

> *The interests and welfare of the human being shall prevail over the sole interest of society or science.*
>
> — Oviedo Convention, Art. 2

> *International treaties which have been ratified in accordance with the constitutional procedure, promulgated and having come into force with respect to the Republic of Bulgaria, shall be part of the domestic law of the land. They shall supersede any domestic legislation stipulating otherwise.*
>
> — Constitution of the Republic of Bulgaria, Art. 5(4)

Key articles: 1 dignity · 3 equitable access · 4 professional standards · 5 consent · 6 persons unable to consent · 8 emergency · 9 previously expressed wishes · 10 private life and information.

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Which rules bind a physician in Bulgaria?

Level	**Main acts for the physician**
Constitution (1991)	human dignity, personal inviolability, private life, right to health care; no medical experiments without voluntary written consent (Art. 29(2))
Ratified international treaties	ECHR; **Oviedo Convention** — take precedence over conflicting domestic law (Constitution, Art. 5(4))
Acts of Parliament	**Health Act** (2004): patient rights, informed consent, research, reproduction; **Medical Establishments Act**; **Health Insurance Act**; **Act on the Professional Organisations of Physicians and Dentists** (1998); Criminal Code, Obligations and Contracts Act (liability)
EU law (a separate legal order)	takes precedence over conflicting national law within its scope; e.g. GDPR (health data, Art. 9), Regulation (EU) No 536/2014 on clinical trials
Secondary legislation	ordinances of the Minister of Health, medical standards
Professional rules	**Code of Professional Ethics of Physicians in Bulgaria**; rules of good medical practice
Institutional rules	internal rules of the hospital — subordinate to all of the above

The Constitution: three provisions every physician should know

> *No one shall be subjected to medical, scientific or other experiments without his voluntary written consent.*
>
> — Constitution of the Republic of Bulgaria, Art. 29(2)

> *The private life of citizens shall be inviolable. Everyone shall be entitled to protection against any illegitimate interference in his private or family life and against encroachments on his honour, dignity and reputation.*
>
> — Constitution of the Republic of Bulgaria, Art. 32(1)

> *No one shall be subjected to forced medical treatment or sanitary measures except in cases provided for by law.*
>
> — Constitution of the Republic of Bulgaria, Art. 52(4)

Each of these is developed in a later class: research (Class 6), confidentiality (Class 3), consent and refusal (Class 2).

The Health Act: the patient as a holder of rights

> *A patient is any person who has sought or is receiving medical care.*
>
> — Health Act, Art. 84(1)

> *Health care is provided regardless of age, sex, origin, language, nationality, race, political affiliation, education, beliefs, cultural level, sexual orientation, personal, social or financial status, disability, or the type and cause of the disease.*
>
> — Health Act, Art. 85 (abridged)

Art. 86(1) — selected rights: respect for civil, political, economic, social, cultural and religious rights; accessible, quality care; more than one medical opinion; protection of health data; clear and accessible information; safety of procedures; prevention of pain and suffering; access to the medical record.

Art. 93: complaints to the Regional Health Inspectorate, which checks within 7 days. **Art. 94:** the patient must also care for his health and not harm the health of others.

The Bulgarian Medical Association and professional self-regulation

Act on the Professional Organisations of Physicians and Dentists (1998):

- every practising physician is a member of the **Bulgarian Medical Association** (BMA) and entered in the register of a regional college (Arts 3–4);
- the BMA drafts the **Code of Professional Ethics** and controls its observance (Art. 5(1)(3)); it adopts **rules of good medical practice**, approved by the Minister of Health (Art. 5(1)(4));
- complaints are examined by the **ethics commissions** of the regional colleges (Art. 39); the penal decree is issued by the chair of the college's board (Art. 40(3)).

Sanctions for breach of the Code (Art. 38)

1. reprimand;
2. fine of one to five minimum monthly wages;
3. removal from the register of the college for 3 months to 1 year.

The penalty does not exclude criminal, civil or labour-disciplinary liability (Art. 42(1)). Historical note: the BMA was founded in 1901, dissolved in 1949, re-established in 1990 and given its statutory framework by this Act in 1998.

The Code of Professional Ethics of Physicians in Bulgaria

Adopted by the congress of the BMA and published in the State Gazette (No. 79/2000, amended No. 85/2013); **binding on every physician**.

Values (Art. 2): competence and responsibility; honesty, humanity, mercy; conscientiousness; tact; politeness; a psychological approach to the sick; tolerance of religious, ethnic and political convictions; **equal treatment** of patients of different social and financial status.

> *Regardless of specialty, qualification or commitments, the physician is obliged to provide emergency care to every patient whose health and life are in immediate danger.*
>
> — Code of Professional Ethics, Art. 11

Art.	Rule (abridged)
10	recognises patients' rights as in the WMA Declaration of Lisbon (1981)
12	may refuse care where trust is lacking — not in an emergency; may refuse under direct threat to own life and health (12(2))
26	no procedures without consent, except immediate threat to life — read with Health Act Arts 87–90: consent comes from the persons designated by law, and the exception needs the statutory conditions (Art. 89(2))
28	right to information and to a second opinion
29	if the patient refuses, the physician explains the prognosis without treatment — unless he declines this information
30	the physician has no right to end the patient's life
32	may refuse, on conscience, to terminate a normal pregnancy

One act — different kinds of liability

Disciplinary

towards the employer, under the Labour Code

Administrative

incl. professional-body sanctions: BMA reprimand, fine, removal from the register (Arts 37–42); complaints and inspection by the Regional Health Inspectorate (Health Act, Art. 93), sanctions under the specific penal provision

Criminal

e.g. negligent bodily harm or death in the exercise of a profession (Criminal Code)

Civil

damages to the patient: contractual or in tort (Obligations and Contracts Act)

Ethics is not only "soft": in Bulgaria the core of medical ethics is codified, and its breach has legal consequences. But meeting the legal minimum is not the same as being a good doctor.

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A structured method in eight steps

1. **Facts** — medical (diagnosis, prognosis, options, uncertainty), the patient's values, the context, conflicts of interest and the applicable law.
2. **The ethical question** — formulated from the position of the decision-maker.
3. **Stakeholders** — patient, family, team, other patients, society.
4. **Principles and duties** — which are in tension, and why?
5. **Options** — more than two: compromise, time-limited trial, second opinion, ethics consultation.
6. **Weighing** — benefits, harms, fairness, legality of each option.
7. **Decision and justification** — the reasons that make one option weightier.
8. **Communication and review** — how to tell the patient and family; what would change the decision.

Three common errors

treating ethics as arithmetic · letting one principle automatically win (the "autonomy trap" or the "paternalist trap") · confusing "legal" with "ethical"

The four-box method (Jonsen, Siegler & Winslade)

1 · Medical indications (beneficence, non-maleficence)

diagnosis and prognosis; goals of treatment; probability of success; plan if treatment fails

2 · Patient preferences (autonomy)

has the patient been informed and does he understand? Capacity? Earlier wishes? Who decides if he cannot?

3 · Quality of life

expected functioning, symptoms and well-being as evaluated with the patient; biases of the evaluator

4 · Contextual features (justice, law)

family, finances, religion, resources, law, conflicts of interest of the clinicians

Both methods lead to the same place: an ethically defensible decision, considering all affected parties, that can be **explained publicly**. Help: clinical ethics consultation or a committee with that function, where available in the hospital; legal advice.

Worked example: an adult refuses a blood transfusion

Case

A 42-year-old Jehovah's Witness with capacity is bleeding after surgery. He refuses blood and blood products, informed of the risk of death; his wife supports him.

	Autonomy	**Beneficence**	**Non-maleficence**	**Justice**
Patient	informed, voluntary, repeated refusal	care consistent with his values	knows the risk of death	equal respect and effort
Physician	verifies capacity, absence of pressure, documents	maximises blood-free options	avoids unwanted invasion of the body	no withdrawal of care because of the refusal
Family	respects his decision	wants him to survive	fears losing him	heard, but does not decide for him

Model conclusion: a capable adult may refuse at any time (Health Act, Art. 90(1)); the refusal is documented with signatures (Art. 90(2)); the team uses the clinically indicated options the patient individually accepts and does not abandon him (erythropoietin and iron do not quickly restore oxygen-carrying capacity). A clear, specific and current refusal of a capable patient must be protected — urgency alone does not cancel autonomy (ECtHR, Pindo Mulla v. Spain [GC], 17.09.2024). Where refusal and a threat to life coincide, the **head of the establishment** may decide on life-saving treatment (Art. 90(4)) — a different situation from a patient who **cannot** consent (Art. 89(2)). How this power relates to a capable refusal and to Oviedo Art. 5 is analysed in Class 2.

Case 1 · “Don’t tell my mother”

The case

A 75-year-old woman has a benign but complex abdominal mass that needs surgery. Her son asks the general practitioner not to tell her: she is very anxious, has had panic attacks, and “in our family the children handle serious news”. The patient trusts her son, but has always expected her doctor to speak to her directly.

1. Whose right is it to decide how much the patient knows — hers or her son’s?
2. How would a deontologist and a care ethicist approach the son’s request?
3. Where is the line between compassionate, staged communication and deception?
4. What does Bulgarian law say about the patient’s right to information and her right to decline it?

Case 2 · The last intensive-care bed

The case

During a severe influenza season one ICU bed remains. Patient A, 82, has advanced COPD; patient B, 45, previously healthy, father of three, has severe post-influenza pneumonia. With intensive care, the estimated (hypothetical) probability of survival to hospital discharge is 30% for A and 80% for B; without it, both are likely to die. Patient A arrived ten minutes earlier.

1. Which criteria of justice are relevant: need, expected benefit, age, "social value", order of arrival?
2. How would a utilitarian and a deontologist decide — and what would each overlook?
3. Is using age or expected quality of life as a criterion fair, or is it discrimination?
4. What do the hospital and the doctor owe to the patient who does not get the bed?

Case 3 · A double dose without established harm

The case

A junior doctor notices that his consultant prescribed twice the recommended dose of an intravenous antibiotic; two doses have already been given. Renal function is normal, the patient has no symptoms, and recovery appears unaffected. The consultant says: "No harm done. Don't document it and don't tell the patient — it will only worry her and invite a complaint."

1. Which principles and which professional virtues are at stake?
2. Is there a duty to tell a patient about an error that caused no harm?
3. Does the hierarchy excuse the junior doctor if he remains silent?
4. What should he do now — clinically (correct, assess, monitor) and through which channels?

Summary

Concepts and theories

1. Ethics is critical reflection on morality; law sets enforceable requirements that ethics can support, exceed or criticise. Medical deontology is the doctrine of professional duties.
2. Consequentialism, deontology, virtue and care ethics each reveal different morally relevant features.
3. The four principles are **prima facie**: they must be specified and openly balanced.

History and law

1. Nuremberg made **voluntary consent** the cornerstone; Geneva, Helsinki and Lisbon developed it; Oviedo created binding treaty obligations for its parties.
2. In Bulgaria the Constitution, Oviedo, the Health Act and the Code of Professional Ethics bind every physician; the BMA enforces the Code.
3. A good decision follows a method and can be justified publicly.

Next class: informed consent — the Health Act, Arts 87–92, in detail, and twelve cases.

Thank you for your attention!